MOCK-UP for discussion (research/33, option C), not the product. Measured: the tamponade cue-card run, kit round A3.1. † = illustrative.

DEBRIEF · FACILITATOR · SIMULATED CASE

Stab wound: tamponade, PEA and pericardiocentesis

M 30 y 80 kg · 11:29 simulated (3 min 36 s on the clock) · 18 October 2026 · Run by: Showcase 2

Patient Monitor Engine
teaching simulator

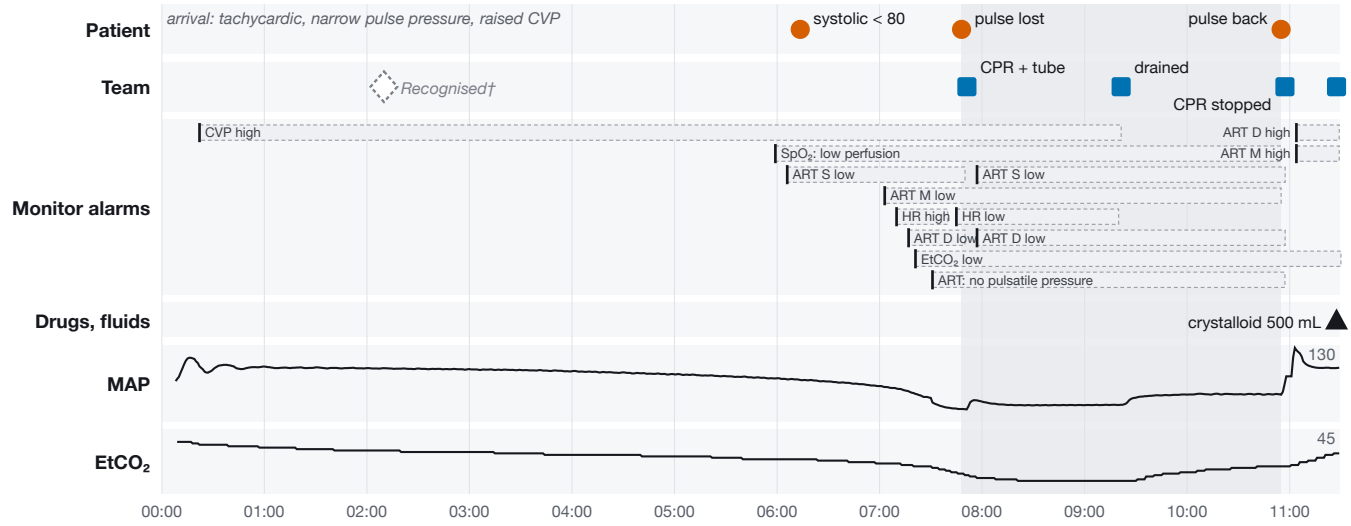
Key times

intervals from the moment named; † not recorded in rc.1

RECOGNITION (INSTRUCTOR MARK)	RECOGNITION → FIRST TEAM ACTION	NO-FLOW: PULSE LOST → CPR	CPR → PERICARDIOCENTESIS
2 min 10 s†	***5 min 41 s†***	**3 s**	**1 min 30 s**
arrival → "tamponade" named · asked ≤ 3 min	*02:10†* → 07:51 (CPR)	07:48 → 07:51 · asked ≤ 10 s	07:51 → 09:21 · asked ≤ 2 min

The run, lane by lane

● patient ■ team ◇ recognised† · alarm bars: raise measured | , end illustrative



Objectives, in behavioural words

observed against expected; it reads the run, it does not grade the person

○ Instructor

Named the tamponade from the monitor

Observed at *02:10†* (instructor mark), *2 min 10 s†* after arrival; asked within 3 min.

✓ Observed

Started CPR after the pulse loss

3 s after the pulse loss (07:48 → 07:51); asked within 10 s.

3 s of 10 s

✓ Observed

Drained the pericardium during CPR

1 min 30 s into CPR (07:51 → 09:21); asked within 2 min.

1 min 30 s of 2 min 00 s

✓ Observed

Stopped CPR when the pulse returned

2 s after the pulse returned (10:55 → 10:57); asked within 30 s.

2 s of 30 s

◆ Gap to explore

Drain before the pulse loss, while the systolic is above 60

Not observed: the pulse was lost at 07:48; drainage came during CPR, at 09:21. Systolic below 80 from 06:13.

† Illustrative: not recorded in rc.1. Option C would record it; the value shown is invented for the layout, made consistent with the measured MAP where one exists.

Key moments and what the monitor showed

HR, MAP, SpO_2, $EtCO_2$ measured; SBP, DBP, CVP and the recognition row illustrative†

	Time	Who	What happened	HR	*SBP†*	*DBP†*	MAP	*CVP†*	SpO_2	$EtCO_2$
1	*02:10†*	◇ Team	*Recognised: "tamponade" named (instructor mark)†*	105	*96†*	*78†*	90	*17†*	99	30
2	06:13	● Patient	Deteriorating: systolic below 80	136	*84†*	*64†*	71	*19†*	99	24
3	07:48	● Patient	Pulse lost (the model) · PEA	46	*26†*	*18†*	21	*22†*	99	14
4	07:51	■ Team	CPR, ventilated with FiO_2 1.0	44	–	–	20	–	99	13
5	09:21	■ Team	CPR: pericardium drained, catheter left in	107	*42†*	*21†*	28	*12†*	99	9
6	10:55	● Patient	Circulation returned (the model)	135	*66†*	*35†*	45	*10†*	99	19
7	10:57	■ Team	Return of circulation: ventilated, catheter in the pericardium	119	*92†*	*49†*	63	*9†*	99	19
8	11:27	■ Team	Post-ROSC: 500 mL crystalloid running	64	–	–	90	–	99	28

† Illustrative: not recorded in rc.1. Option C would record it; the value shown is invented for the layout, made consistent with the measured MAP where one exists.

Debrief hints for this scenario

from the scenario file placeholder wording, not reviewed by the owner

What this case is built to show

- Tamponade is an obstructive shock: tachycardia, a narrow pulse pressure and a raised CVP come before the arrest.
- Induction drugs can remove the compensation that keeps the pressure up.
- In PEA from tamponade, compressions alone do not refill the heart: decompression is the treatment.

Openers for the analysis (advocacy-inquiry)

- "I saw the systolic go below 80 at 06:13 [moment 2]. I'm curious what you were weighing then."
- "The drainage came 1 min 30 s into CPR [moment 5]. What would have let it come before 07:48?"

Choosing a mode for each gap

PEARLS analysis; the author suggests, the facilitator decides

Gap or strength	Domain	Rationale evident?	Suggested mode
◆ Drain before the pulse loss	cognitive (anticipation)	no	focused facilitation (advocacy-inquiry)
✓ CPR within 3 s of the pulse loss	technical / team	yes	learner self-assessment: what made it fast?
✓ CPR stopped 2 s after the pulse returned	cognitive (re-evaluation)	yes	brief directive: the arterial line and capnogram cue

Next pages (facilitator): trends with the same markers; drugs, fluids and the full timeline; bookmarks; the one-row-per-minute appendix with SBP, DBP and CVP columns. Learner handout: printed separately, overleaf.

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LEARNER HANDOUT · YOUR CASE · NO MARKS, NO VERDICTS

Stab wound: tamponade, PEA and pericardiocentesis

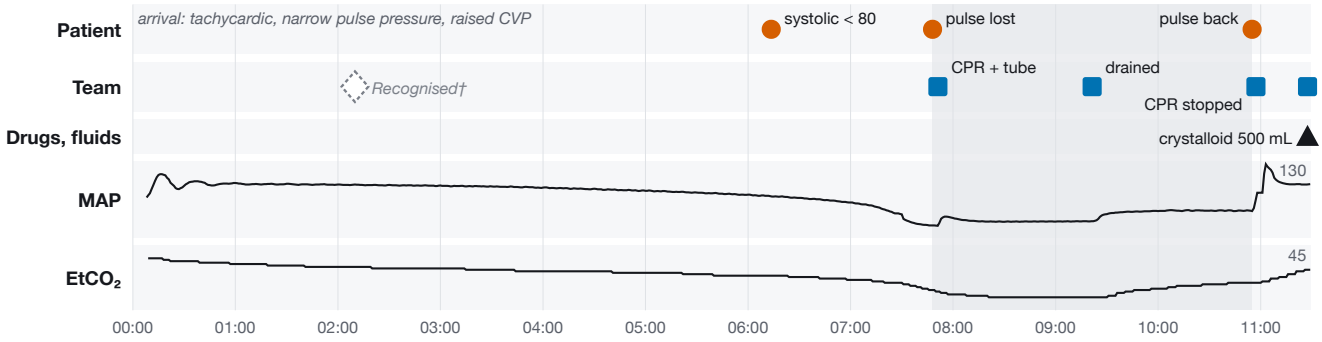
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This sheet is yours to keep. It shows what happened and when, so you can think it through; the conversation, not this page, is the debrief.

Your run at a glance

patient events ● and team actions ■ on one time axis



Key moments

02:10†	*Recognised: "tamponade" named (instructor mark)†*
06:13	Deteriorating: systolic below 80
07:48	Pulse lost (the model) · PEA
07:51	CPR, ventilated with FiO_2 1.0
09:21	CPR: pericardium drained, catheter left in
10:55	Circulation returned (the model)
10:57	Return of circulation: ventilated, catheter in the pericardium
11:27	Post-ROSC: 500 mL crystalloid running

Questions to think about

write a line or two before the debrief

At 06:13 the systolic fell below 80. What did you notice on the monitor, and what did you make of it?

The pulse was lost at 07:48 and the pericardium drained at 09:21. What would have let you drain earlier?

One thing you will keep doing, one thing you will do differently.

Take-home and next steps

placeholder wording from the scenario hints

- Narrow pulse pressure + raised CVP + tachycardia after chest trauma: think tamponade, plan drainage early.
- Before induction in a compensating patient, ask what keeps the pressure up.
- Practise: run this case again from the bookmark at arrival and aim to drain before the pulse loss.

† Illustrative: not recorded in rc.1. Option C would record it; the value shown is invented for the layout, made consistent with the measured MAP where one exists.